

Premature birth

“Premature” describes both the time of birth, before 37 weeks, and how well prepared your baby is for life outside of the womb. In the US, approximate 10 percent of births occur preterm.

Your baby may be born prematurely either because an early delivery is advised on medical grounds (see below), or you go into spontaneous preterm labor. The earlier the birth, the higher the chance of complications in the baby, such as breathing problems or infections. Nowadays, however, huge advances in the care of premature babies means that babies born as early as 23 or 24 weeks may survive. If your baby is premature, they may need to spend time in a [neonatal intensive care unit \(NICU\)](#).

Advising an early delivery

An early delivery may be recommended if the mother’s or baby’s health is in danger, for example, if the mother has a medical problem, such as a heart condition that could increase her physical stress, or [preeclampsia](#), which could endanger her own and her baby’s health. An early delivery may also be advised if a scan shows that the placenta is not functioning well and the baby is not receiving enough oxygen. Forty-seven percent of babies in the US delivered before 32 weeks are born by Cesarean, as are about a third of babies born between 34 and 36 weeks.

Spontaneous preterm labor

The cause of spontaneous labor before 37 weeks is often unknown. However, it is more likely if a woman has a major abnormality of the uterine wall, such as large fibroids (see [Fibroids during pregnancy](#)); a weakness in the cervix, which may have been present from birth or have occurred after surgery to the cervix; or is pregnant with multiples. Infections like bacterial vaginosis can also set off contractions at an early stage.

What might be done? Preterm labor can’t be stopped, but medication can slow the process and reduce some risks.

A course of steroid injections, given about 24 hours apart, is one of the most important treatments to reduce the risks associated with preterm delivery for your baby. The maximal benefit of steroid use is 2–7 days after the initial dose, so it will only be recommended if you are at risk of preterm delivery within 7 days.

You may be given an injection to reduce the frequency of contractions. This can prolong pregnancy for a few days, during which time the steroids can take effect. Also, if necessary, you can be transferred to a hospital with NICU facilities.

You may be advised to have injected antibiotics since preterm babies are susceptible to bacterial infections caught via the cervix during birth.